

THE RATE SHOCK SURVIVAL GUIDE

Your Premiums Just Jumped. Here's *Every Option* You Have.

A plain-language walkthrough of every realistic path forward when your health insurance renewal comes in 10, 20, or 30% higher than last year. Written for CEOs, CFOs, founders, and HR leaders who are ready to see all their options.

10–30%

Typical annual premium increases for small and mid-size businesses

7+

Distinct options available (most businesses only hear about 1 or 2)

10–500

Employee companies with the most to gain from alternatives

You're Stuck Inside a System That Isn't Built for You

You just opened your renewal letter. The number is higher than last year. Maybe a lot higher. Your first reaction is frustration. Your second is a question: what can I actually do about this?

If you're on a **fully funded health insurance plan** (the standard model where you pay a fixed monthly premium and the carrier assumes all claims risk), the honest answer is: not much. And that's the real problem.

The Fully Funded Trap

Fully funded plans are the default in American health insurance. They're simple, familiar, and easy for brokers to administer. But they come with structural limitations that cost your company real money every year.



You don't own your claims data.

The carrier owns data about how your employees use benefits. You're making a six- or seven-figure spending decision every year with almost no information about what you're paying for.



The carrier keeps everything.

You pay the same premium whether claims are \$50,000 or \$500,000. If your employees are healthy and claims are low, the carrier keeps the difference. You get nothing back.



You have no strategic levers.

Your only options at renewal: accept the increase, shift costs to employees, or shop for a slightly cheaper version of the same thing. There is no mechanism to improve your position from the inside.



No one helps your employees navigate care.

Employees navigate the healthcare system alone. No help finding the right provider, understanding bills, or avoiding unnecessary costs. That lack of guidance drives claims higher, which drives your premiums higher.

This system isn't broken by accident. It's optimized for the status quo. It's easy for carriers, easy for brokers, and expensive for you. The good news: there are proven alternatives. The bad news: your current broker may not be showing them to you.

Why Your Broker Might Not Be Showing You These Options

This isn't about bad people. Most brokers are decent professionals doing their jobs. But the traditional brokerage model creates blind spots that cost you money. Here's why the alternatives in this guide rarely come up in a standard renewal conversation.

1

Level-funded plans require more work.

Quoting a fully funded plan takes a few minutes. Analyzing whether a company is a good fit for a level-funded or partially self-funded plan takes real underwriting work, claims analysis, and ongoing management. Many brokers aren't set up for this, and their compensation doesn't reward the extra effort.

2

Most brokers see PEOs as competition, not as an option for you.

A Professional Employer Organization pools small businesses together for better rates and broader benefits. But many brokers lose the account entirely when a client joins a PEO. Instead of recommending what might be best for you, the incentive is to steer you away from it. A broker who partners with PEOs is putting your interests first.

3

ICHRAs are relatively new, and many brokers don't understand them.

Individual Coverage Health Reimbursement Arrangements let employers give employees a fixed allowance to buy their own coverage. It's a powerful option for diverse workforces. But it's a different model that requires different expertise. Brokers who built their practice on group plans may not have the knowledge or tools to offer it.

4

The traditional model rewards the status quo.

Broker commissions on fully funded plans are straightforward and predictable. Alternative structures sometimes pay differently. When the easy path also happens to be the most profitable one for the broker, the alternatives rarely make it into the conversation.



Level-Funded

Requires real underwriting
analysis most brokers
won't do



PEO

Seen as competition
by most brokers, not
offered as an option



ICHRA

Too new for most brokers
to understand or
know how to offer

*"If the only response you get to a 20% rate increase is
'we'll shop it to a few carriers,' you're only seeing a fraction of the market."*

— Jonathan Engle, Founder, Kingsfoil Health

Seven Paths Forward When Premiums Spike

Every company facing a rate increase has more options than they've been shown. Here are seven realistic paths, starting with the most accessible and moving toward structural changes with more upside.



OPTION 1

Negotiate with Your Current Carrier

Your renewal rate is not final. It's an offer. Carriers have room to adjust, especially if your group is healthy, you've been loyal, or you're willing to adjust plan design. Ask for a detailed breakdown of how the renewal was calculated and what portion is claims versus market trends. Push for a multi-year rate guarantee. Negotiation alone rarely closes a 20%+ gap, but it can take a few points off the top.

Best for: Healthy groups wanting to stay with their current carrier



OPTION 2

Shop Competing Fully Funded Carriers

Send your census data to competing carriers and compare quotes. You may find better rates for your demographics, or a carrier running promotional pricing. Potential savings of 5-15% when switching carriers, though results vary. This is the most common response and sometimes it's enough. But you're still inside the same model. If costs keep rising, your premiums will keep rising with them.

Best for: Companies wanting simplicity, primarily shopping on price



OPTION 3 — WHERE THE REAL OPPORTUNITY BEGINS

Level-Funded Plans (Partial Self-Funding)

A **level-funded plan** looks and feels identical to traditional insurance for your employees: same ID cards, same networks, same claims process. But the financial structure is different. Your premium is split into three buckets: administration, a stop-loss policy that caps catastrophic risk, and a claims fund. If your group's actual claims come in below the fund, you can receive a refund of unused dollars. You also get access to your claims data, proactive claims management, and a concierge experience that helps employees navigate care. Your downside is capped. Your upside is real.

Best for: 25-500 employee companies, especially healthy groups on fully funded plans

Why level-funded is where the conversation changes: Options 1 and 2 are adjustments within the fully funded model. Starting with Option 3, you're changing the underlying financial structure, which is where the real opportunity for cost control, data transparency, and better employee experience begins.



OPTION 4 — ESPECIALLY STRONG FOR SMALL COMPANIES

PEO Partnership

A **PEO (Professional Employer Organization)** pools multiple small businesses together to access benefits, HR support, payroll, and compliance services normally reserved for large companies. Instead of your 30-person company buying insurance alone, your employees join a pool of thousands. That larger pool gets better rates, broader networks, and more plan options. PEOs also handle HR administration, payroll, and compliance, which is a major advantage if you don't have a dedicated HR team.

Best for: 10-100 employee companies wanting big-company benefits + bundled HR/payroll



OPTION 5

ICHRA (Individual Coverage HRA)

An **ICHRA** lets you give each employee a fixed monthly allowance to buy their own individual health insurance plan. Employees pick coverage tailored to their age, location, and family. You set a predictable budget. A 25-year-old single employee and a 55-year-old with a family of four have very different needs. ICHRA lets them each choose accordingly, with tax-free reimbursement up to your set allowance.

Best for: Diverse workforces, remote teams, companies wanting predictable budgets



OPTION 6

Strategic Plan Design Changes

Sometimes the answer isn't a new carrier or structure. It's a smarter version of what you have. Raise the deductible and offset it with an **HRA** (employer-funded medical expense account) or **HSA** (employee-owned tax-advantaged savings account). Both put money in employees' hands to cover the higher deductible, while lowering your premium. A good interim step while evaluating bigger changes.

Best for: Companies not ready for structural changes, wanting savings this cycle



OPTION 7 — ADVANCED STRATEGY

Reference-Based Pricing

Reference-based pricing (RBP) pays for medical services based on a set reference rate (often a percentage of Medicare) rather than whatever the hospital charges. The same procedure can cost 3x more at one facility than another with no difference in quality. RBP addresses this directly. It requires strong member advocacy and experienced management, but the potential savings are significant for companies willing to take a more active approach.

Best for: Larger companies comfortable with hands-on management, seeking substantial savings

The key takeaway: You don't have to pick just one. The most effective strategies often combine multiple approaches. A level-funded plan with strategic HRA design. An ICHRA for remote workers alongside a group plan for HQ staff. The right answer depends on your company. The important thing is that someone runs the full analysis.

Comparing Your Options at a Glance

This table compares the four primary plan structures. Use it as a starting point. The right answer depends on your company's size, your group's health profile, and how much change you're comfortable with.

FEATURE	FULLY FUNDED	LEVEL-FUNDED	PEO	ICHRA
Best company size	Any	25–500 employees	10–100 employees	Any
Employee experience	Standard	Same + concierge support	Big-company options	Individual plan choice
Claims data access	✗ No	✓ Yes	Varies by PEO	N/A
Refund in low-claims year	✗ No	✓ Yes	✗ No (lower base rates)	N/A (fixed allowance)
Catastrophic risk protection	Carrier absorbs	Stop-loss policy caps exposure	PEO manages pooling	Individual plan
Proactive claims management	✗ Rare	✓ Built in	Varies	Depends on plan
Budget predictability	Fixed premium	Level monthly + refund upside	Bundled per-employee rate	Fixed monthly allowance
HR/admin burden	Low	Low (with good broker)	Very low (PEO handles)	Moderate
Bundled HR/payroll	✗ No	✗ No	✓ Yes	✗ No
Commission transparency typical	✗ Rarely	Depends on broker	Varies	Depends on broker

These options are not mutually exclusive. Many companies combine strategies. For example, a level-funded plan with strategic HRA/HSA design, or an ICHRA for part-time employees alongside a group plan for full-time staff. The right answer is often a combination, not a single product.

\$0

What most businesses receive back from fully funded premiums in a healthy year

2nd

Health insurance is typically the second-largest expense after payroll

1 in 3

Approximate share of small businesses that have explored alternatives to fully funded

Which Path Fits Your Company?

Start with these three questions. Your answers will narrow the field to the most realistic options for your situation.

How many employees are on your health plan?



10–50 employees

PEO and **ICHRA** are your strongest options. **Level-funded** may also be available depending on group health.

51–250 employees

Level-funded is your sweet spot. **PEO** or **ICHRA** may also fit. You have the most options here.

250–500 employees

Level-funded and partial self-funding offer the greatest potential. **Reference-based pricing** becomes viable.

How would you describe your group's overall health?



Generally healthy

Level-funded gives you the most upside. A low-claims year means potential refunds. This is where healthy groups win.

Average or mixed

Level-funded + claims management can improve your profile over time. **PEOs** spread risk across a larger pool.

High utilizers

PEOs (risk pooling), **plan design changes** (HRA/HSA offsets), and **concierge support** are your best levers.

How much change is your organization ready for?



Minimal change

Start with **negotiation + plan design**. Create savings without disrupting the employee experience.

Open to a shift

Level-funded, PEO, or ICHRA offer more upside. A good broker manages the transition for you so your team barely notices.

No single path is right for every company. The important thing is that you see all the options before you decide. A thorough analysis considers your size, your claims profile, your budget, and your team's needs, then recommends the combination that fits.

This Is the Analysis We Run for Every Client

Everything in this guide is a condensed version of the work we do for every company we serve. We don't start with a product. We start with your situation, your numbers, and your goals. Then we show you every option that fits.

If your current broker is only showing you fully funded quotes, you're seeing a fraction of the market. Here's what working with Kingsfoil Health looks like.



We show you all the options.

Fully funded, level-funded, PEO, ICHRA, and plan design strategies. We analyze every viable structure for your company. You see the full picture before making a decision.



We tell you what we earn.

We disclose our compensation on every option, in writing, before you decide. If two plans are equivalent for your company, you should know whether one pays us more than the other. Transparency isn't a policy for us. It's a principle.



We partner with PEOs instead of fighting them.

If a PEO is the best option for your company, we'll recommend it and help you get set up. Our job is to find you the best outcome, not to protect our commission.



We stay after the sale.

Year-round support. Help for your employees navigating the healthcare system. Plan performance reviews so you're never blindsided at renewal again. We're your advocate inside a system that isn't built for you.

See All Your Options

We run this analysis for every client, free, no obligation. If you're facing a rate increase and want to know what's really available to your company, start the conversation.

kingsfoilhealth.com/talk-to-us

HONOR

EXCELLENCE

ACCOUNTABILITY

RESPECT

TRUST